

Interview Transcript with Dr. Sarah Johnson (Seattle, WA) - Primary Care Physician, Community Health Network

Interviewer: Thank you for speaking with us, Dr. Johnson. Could you introduce yourself and describe your role during the COVID-19 pandemic? Dr. Johnson: Thank you for having me. I'm Dr. Sarah Johnson, a primary care physician working within a community health network in the Seattle metropolitan area. I've been in family and community medicine for twelve years, and during the pandemic my role pivoted dramatically to include telephone and video triage, setting up remote-monitoring programs for patients isolating at home, coordinating with community health workers for outreach to vulnerable populations, and helping to design our clinic's vaccination strategy when vaccines became available.

Interviewer: Seattle was one of the early U.S. hotspots. What were your first encounters with COVID-19 like in the outpatient setting? Dr. Johnson: Our clinic experienced an abrupt shift in February March 2020. We had an early cluster in some long-term care facilities and then community spread. The initial weeks were chaotic; clinic visits dropped precipitously as patients avoided healthcare facilities, and we needed to rapidly determine which services could be delivered remotely. Initially, telehealth infrastructure was minimal, and we scrambled to implement phone-based workflows for triage. Many of our medically vulnerable patients lacked access to reliable internet or devices, so much of the early work involved creative outreach - telephone, home visits when necessary, and coordination with community organizations to get pulse oximeters and thermometers to those who needed them.

Interviewer: How did primary care adapt in terms of clinical workflows? Dr. Johnson: We split workflows into respiratory and non-respiratory tracks to minimize cross-infection. Our nurses performed phone triage and used standardized protocols to decide who needed in-person evaluation versus home monitoring. We established "respiratory clinics" in separate areas or at alternate sites for testing and in-person assessments. Telemedicine became central - but it also exposed inequities. We created hybrid approaches: phone-based monitoring paired with community health worker check-ins for patients without broadband access. Remote monitoring with pulse oximeters was a game changer for managing mild to moderate cases at home, with clear thresholds for escalation.

Interviewer: What were the major challenges for vulnerable populations in your area? Dr. Johnson: Inequities were glaring. Patients facing housing insecurity, limited English proficiency, or precarious employment were at higher risk of exposure and had fewer resources to isolate. Many essential workers couldn't afford to miss work, which increased household transmission. Additionally, undocumented patients were hesitant to seek testing or care due to fears about cost or immigration consequences. Addressing these gaps required community partnerships, culturally competent messaging, and mobile outreach clinics that provided testing and vaccination without onerous documentation requirements.

Interviewer: How did telemedicine change the nature of care you provided? Dr. Johnson: Telemedicine allowed us to maintain continuity for chronic disease management and triage acute symptoms. For many older adults and those with mobility challenges, remote visits were convenient. However, certain assessments - skin lesions, complex physical exams, vaccinations - could not be done remotely. We adopted a blended model: routine follow-ups and counseling by telehealth, and scheduled in-person slots for essential physical care. For remote monitoring, we standardized devices and thresholds - for example, instructing patients to seek emergency care if SpO2 fell below 92% or if they developed new confusion or chest pain.

Interviewer: Testing and contact tracing were critical. How did your clinic interact with public health efforts? Dr. Johnson: We established referral pathways to community testing sites and partnered with public health for contact tracing. Initially, contact tracing was manual and effective when case numbers were low, but it became overwhelmed during surges. We supported community-led outreach teams that could perform culturally sensitive contact tracing and help coordinate services like isolation housing and food assistance for those who needed to quarantine. Collaboration with public health was uneven across jurisdictions, and data-sharing gaps sometimes limited timely interventions.

Interviewer: How did you handle vaccine rollout and hesitancy within your patient population? Dr. Johnson: Vaccine rollout presented logistical and equity challenges. Prioritization frameworks required outreach to elderly and high-risk patients. We set up mass vaccination clinics with community partners and mobile units for homebound patients. Addressing hesitancy required tailored conversations - listening to concerns, correcting misinformation, and using trusted messengers from the community. We found success in deploying bilingual staff and collaborating with faith leaders. Transparent discussion of side effects and clear information about benefits also helped increase uptake.

Interviewer: What were some clinical pearls for managing mild to moderate COVID in the community setting? Dr. Johnson: Early identification of at-risk patients was crucial - those with diabetes, obesity, chronic lung or heart disease, or immunosuppression. Use of home pulse oximetry and establishing clear escalation criteria reduced unnecessary ED visits while ensuring timely transfers for those who needed oxygen or advanced care. Providing social support (food, isolation housing, medication delivery) removed barriers to proper isolation and adherence. We also used anticoagulation guidance and early monoclonal antibody referrals for eligible high-risk outpatients when they became available, which reduced progression to hospitalization in some cases.

Interviewer: How did your practice support staff wellbeing during prolonged surges? Dr. Johnson: Primary care staff experienced moral distress too - watching patients suffer and worrying about bringing infections home. We implemented rotating schedules, capped the number of high-intensity teletriage shifts, and offered paid time off for rest. We also created peer-

support groups and brought in counselors for staff-led sessions. Protecting administrative and nursing staff with clear PPE guidance and minimizing unnecessary exposure was also key to maintaining operations.

Interviewer: What technological or operational innovations from the outpatient setting do you think should be retained post-pandemic? Dr. Johnson: Several innovations are worth retaining: telemedicine for routine care, remote monitoring programs for chronic disease and acute respiratory illness, hybrid models combining community health workers with clinical teams, and streamlined communication channels with public health. Importantly, reimbursement parity for telehealth and investments in broadband and device access for vulnerable populations will be necessary to make these sustainable and equitable.

Interviewer: How did misinformation affect your patient relationships, and how did you combat it? Dr. Johnson: Misinformation - about treatments, vaccines, and the severity of disease - undermined trust. Combating it required proactive outreach: social media engagement, partnerships with local radio, translated materials, and one-on-one discussions during visits. We emphasized empathy - acknowledging fears while providing clear evidence-based guidance, and we leveraged community leaders who had credibility.

Interviewer: Can you share a memorable patient story that influenced clinic policy or outreach? Dr. Johnson: A memorable case involved a multigenerational household where the primary breadwinner worked in a crowded setting. Several family members fell ill, and the lack of paid sick leave meant delayed testing and isolation. After that incident, we prioritized outreach to workplaces with cluster risks and coordinated with local employers and unions to improve sick-leave policies. That case highlighted how social determinants were central to outbreak control and shaped our advocacy efforts with local policymakers.

Interviewer: Beyond acute care, what longer-term consequences have you observed in primary care? Dr. Johnson: We saw worsening control of chronic conditions due to deferred care - higher A1c levels, uncontrolled hypertension, delayed cancer screenings. The mental health burden increased sharply: anxiety, depression, substance use, and family stress. Primary care clinics needed to expand behavioral health integration and invest in proactive outreach to close care gaps created during the pandemic.

Interviewer: What policy changes would you recommend to strengthen primary care for future public health emergencies? Dr. Johnson: Strengthen funding for community health centers, ensure sustained reimbursement for telehealth, invest in broadband and remote device access for low-income patients, and formalize partnerships between primary care and public health for rapid mobilization. Paid sick leave policies and worker protections are also public health measures that reduce transmission. Finally, build stockpiles of essential outpatient supplies - pulse oximeters, PPE for home visits, and mobile testing kits.

Interviewer: What is your biggest hope coming out of this pandemic for primary care? Dr.

Johnson: My hope is that primary care gets recognized and resourced as a foundation of health systems - not just during crisis but continuously. When primary care is robust, we can prevent many hospitalizations and mitigate the broader social harms of pandemics. The creative solutions we developed - community partnerships, telemedicine, and targeted outreach - should be preserved and scaled equitably.

Interviewer: Thank you for this comprehensive account, Dr. Johnson. Any final thoughts? Dr.

Johnson: Thank you. I would stress that health equity must be central to preparedness planning. The pandemic exposed who we leave behind. Investing in social supports, community-based care, and primary prevention will protect communities and healthcare systems alike in the next emergency. --- End of Interview ---